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Integrating Physical Activity Into Routine Psychiatric Care
A Review

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IMPORTANCE Adults with severe mental illness (SMI) face a 10- to 20-year reduction in life expectancy, largely due to heightened cardiometabolic diseases. Low levels of physical activity (PA) and prolonged sedentary behavior (SB) are modifiable risk factors that contribute to this mortality gap. Although strong evidence demonstrates that PA is safe and effective in improving psychiatric and physical outcomes, systematic integration into clinical practice remains limited.

OBSERVATIONS Individuals with SMI are among the most physically inactive groups in society, often spending more than 10 hours per day sedentary and rarely meeting recommended PA recommendations. Low PA and high SB exacerbate neuroinflammatory, neuroendocrine, and cardiometabolic pathways implicated in psychiatric morbidity. Recent meta-analyses show that structured PA, that is, exercise, produces moderate to large reductions in depressive and psychotic symptoms, as well as benefits for cognition, quality of life, and cardiometabolic health. Evidence also suggests that mentally passive SB, such as prolonged television viewing, are associated with poorer mental health outcomes. Physical activity should be individualized to each person's capabilities and preferences, emphasizing forms they find enjoyable. Two sessions of strength training weekly are advised, and greater mental health benefits typically arise from leisure or active travel PA. To guide translation into care, the 5A framework (Ask, Assess, Advise, Assist, Arrange) provides a structured, pragmatic approach: clinicians can screen for PA and SB, assess readiness and safety, provide tailored advice, support motivation and goal setting, and arrange follow-up, referral, and community support.

CONCLUSIONS AND RELEVANCE PA should be considered a core component of psychiatric care. By systematically promoting PA and reducing prolonged SB through structured clinical frameworks, psychiatrists and other mental health professionals can improve symptoms, enhance physical and cognitive health, and help narrow the life expectancy gap experienced by people with SMI.

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Adults with severe mental illness (SMI), such as schizophrenia, major depressive disorder (MDD), and bipolar disorder, face a 10- to 20-year reduction in life expectancy, largely due to the heightened prevalence of cardiometabolic diseases.[1,2] In the general population, there is robust evidence that a lack of physical activity (PA; any bodily movement that increases energy expenditure) and prolonged sedentary behavior (SB; sitting or reclining during waking time) are associated with increased cardiometabolic disorders, physical multimorbidity, and premature mortality.[3-7] To compound matters, adults with SMI are consistently among the most sedentary groups in society.[1,8] For example, individuals with schizophrenia spend 10 hours per day sedentary, and those with depression or bipolar disorder are 30% to 50% less likely to achieve recommended PA levels (**Table 1**[8-12]).[1] These behaviors exacerbate neuroinflammatory, neuroendocrine, and cardiometabolic pathways that contribute to poor outcomes across people with severe mental disorders.[13-16]

Despite robust evidence demonstrating that PA, in particular its structured form, exercise, yields moderate to large benefits for psychiatric symptoms, cognition, and physical health,[17-21] translation into psychiatric practice remains limited.[22] Considering this, the central clinical question addressed in this review is to outline how psychiatrists and other mental health professionals can systematically assess, prescribe, and support increasing PA and reduce SB in adults with SMI.

Methods

This narrative review synthesizes evidence on integrating PA and reducing SB into adult psychiatric care for those with SMI. We prioritized high-level evidence, including umbrella reviews, systematic reviews, and meta-analyses, along with major clinical guidelines published in English between January 2015 and September 2025. Sources included PubMed/MEDLINE, PsycINFO, and Cochrane

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Table 1. Physical Activity and Sedentary Behavior in Adults With Severe Mental Illness

Disorder	Meeting WHO PA guidelines, % (95% CI)	Relative risk of not meeting guidelines (vs controls), OR (95% CI)	Sedentary time, mean (95% CI), h/d	Difference in sedentary time vs controls, h (95% CI)	Key adverse outcomes implicated
Major depressive disorder[8-10]	≈40 (30-50)	1.5 (1.1-2.0)	8.0 (7.1-8.9)	1 h (0.8-1.2)	Greater symptom severity, metabolic risk, impaired sleep, cognitive decline
Bipolar disorder[10,11]	20-35 (13-59)	1.3 (0.9-1.8)	8.8 (7.6-10.0)	~1.7 ↑ SB	High obesity and metabolic syndrome prevalence; episodic fluctuations in activity
Schizophrenia spectrum disorders[8,10,12]	<20 (≈15%, 10-20)	1.5 (1.1-2.0)	9.8 (8.7-10.9)	2.80 (1.47-4.1)	Severe cardiometabolic morbidity, reduced functioning, 15-y to 20-y mortality gap

Abbreviations: OR, odds ratio; PA, physical activity; SB, sedentary behavior; WHO, World Health Organization.

Library, supplemented by manual searches of reference lists and guideline repositories. Core search terms combined controlled vocabulary and keywords related to *exercise*, *physical activity*, *sedentary behavior*, and SMI (eg, MDD, bipolar disorder, schizophrenia, psychosis), paired with *meta-analysis*, *systematic review*, *umbrella review*, *guideline*, *implementation*, and *practice*.

Inclusion focused on adult populations (≥18 years old) with SMI, defined as MDD, bipolar disorder, and schizophrenia, that investigated PA or structured exercise interventions. Outcomes included psychiatric, functional, or cardiometabolic outcomes. We excluded pediatric samples and nonclinical populations. When multiple syntheses addressed a topic, we selected the most recent and comprehensive reviews, cross-checking conclusions against major international guidelines.[8,13,23,24] The findings were narratively synthesized.

Observations

Pathophysiology

PA and SB have a multisystem effect on biological systems central to the onset and course of mental disorders. For instance, reduced PA is associated with dysregulation of the hypothalamic-pituitary-adrenal axis, leading to altered cortisol secretion patterns observed in depression.[25-28] Low PA levels also contribute to chronic low-grade inflammation, with elevated circulating C-reactive protein, interleukin 6, and tumor necrosis factor α consistently reported in schizophrenia, bipolar disorder, and major depression.[15,29,30] Low PA also contributes to dysregulation of dopaminergic reward circuitry, which underpins anhedonia and motivational deficits in depression and schizophrenia.[31-33] Regular PA enhances dopamine transmission and striatal function, thereby improving reward sensitivity and motivational drive.[28,33-35] These mechanisms may help explain observed improvements in negative symptoms and motivational impairments in psychiatric populations engaging in structured exercise.[30-32,34-36]

Regular PA exerts anti-inflammatory and neuroprotective effects, reducing systemic inflammatory markers and enhancing neuroplasticity through upregulation of brain-derived neurotrophic factor (BDNF) and hippocampal neurogenesis.[28,37-39] These processes are thought to underlie improvements in mood, cognition, and stress regulation.[28,37] PA also enhances cardiorespiratory fitness and insulin sensitivity, counteracting cardiometabolic dysfunction that disproportionately contributes to premature mortality in psychiatric populations.[2,40,41]

High levels of SB independent of PA may contribute to increased cardiometabolic risk and impaired cerebral perfusion, with SB, particularly that classified as mentally passive (ie, requiring lower cognitive demand such as television viewing), associated with greater depression severity and cognitive decline.[20,42] Emerging evidence suggests that replacing mentally passive SB with light PA or mentally active sedentary behaviors (eg, reading) may attenuate these risks.[10,43]

Clinical Presentation

Low PA and high levels of SB are highly prevalent across psychiatric populations and present with distinct patterns that exacerbate both psychiatric and physical morbidity. Epidemiological studies consistently show that individuals with schizophrenia spectrum disorders are among the least active, with fewer than 20% meeting World Health Organization 2020 PA guidelines and average sedentary time exceeding 10 hours per day.[8,10] Negative symptoms of schizophrenia, cognitive impairment, and antipsychotic-induced sedation compound these patterns.[13]

In MDD, low PA manifests as reduced daily step counts and diminished engagement in leisure-time activity, often accompanied by anhedonia, fatigue, and psychomotor retardation.[17] High SB, particularly mentally passive behaviors such as prolonged television viewing, is associated with greater symptom severity and poorer sleep quality.[20,44]

People with bipolar disorder demonstrate fluctuating PA profiles: depressive phases mirror inactivity seen in MDD, while manic episodes may feature transient increases in PA, often followed by relapse into inactivity.[45] A summary of the evidence is presented in Table 1.

Clinically, these presentations frequently intersect with obesity, metabolic syndrome, and cardiorespiratory deconditioning, which in turn worsen psychiatric prognosis.[1,2] Importantly, individuals' report of fatigue, poor sleep, and cognitive concerns should prompt clinicians to evaluate PA and SB, as these behaviors are both modifiable and clinically relevant.

Emerging longitudinal evidence indicates that early adoption of PA interventions for people with SMI may attenuate the development of cardiometabolic multimorbidity.[2,45,46] Embedding interventions targeting the promotion of PA and reductions in SB early in the course of illness holds potential to not only improve psychiatric trajectories but also reduce the accumulation of physical health conditions that drive excess mortality.[8,47]

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Table 2. Tools for Assessing PA and SB in Psychiatric Populations

Tool	Domains assessed	Time burden	Strengths	Limitations	Clinical use
Physical Activity Vital Sign[48]	Weekly minutes of moderate to vigorous PA	<1 min	Very brief; feasible for routine screening in busy settings	Self-report; limited information on PA intensity or context	"Vital sign" for PA screening in outpatient and community care
International Physical Activity Questionnaire[49]	PA domains (leisure, work, transport, household) and SB	5-10 min	Captures multiple PA contexts; globally validated	Recall bias; cognitive burden for some psychiatric populations	Research and detailed behavioral profiling
Simple Physical Activity Questionnaire[50]	PA, SB, and sleep over the past 7 d	10-15 min	Designed and validated specifically for psychiatric populations; feasible and clinically accepted in SMI	Requires interviewer training; modest recall bias	Gold-standard self-report tool for PA/SB assessment in SMI
Sedentary Behavior Questionnaire[51]	Time spent in specific sedentary domains (TV, computer, reading, transport, hobbies)	5-10 min	Differentiates types of sedentary behavior; validated against accelerometers; useful for targeting mentally passive vs active SB	Self-report; less precise for total sedentary time; potential recall bias	Understanding sedentary patterns, guiding SB-reduction counseling and interventions
Accelerometry, wearable devices	PA, SB, intensity, bout patterns	Continuous	Objective, precise measurement; increasingly affordable	Cost, adherence, and data-processing burden	Research, audits, and selected clinical monitoring
Smartphone-based accelerometers	Steps, PA, SB (via built-in sensors or apps)	Continuous	Ubiquitous, low-cost, scalable; enables passive, real-time monitoring	Accuracy varies by device; data privacy concerns	Scalable population-level monitoring; adjunct to clinical follow-up
Physical Activity Readiness Questionnaire[52]	Safety and medical readiness for PA	2-3 min	Standardized screening for contraindications to exercise	High false-positive rate in SMI; literacy dependent	Safety screening before initiating higher-intensity PA
SB-specific questions	Mentally passive vs active SB	<2 min	Differentiates cognitively engaging vs passive SB; easy to integrate into assessment	Limited validation; subjective estimation	Brief adjunct to PA screening and behavior-change counseling

Abbreviations: PA, physical activity; SB, sedentary behavior; SMI, severe mental illness; TV, television.

Physical Activity and Sedentary Behavior Assessment

Routine assessment of PA and SB is recommended but remains underused in psychiatric practice (**Table 2**[48-52]).[8] The Physical Activity Vital Sign tool provides a brief and validated 2-item screener, estimating weekly minutes of moderate to vigorous PA.[53] Its brevity allows integration into standard psychiatric consultations, with sensitivity approaching 80% for identifying inactivity relative to accelerometer benchmarks,[54] and may help identify individuals with bipolar disorder or schizophrenia at risk of cardiometabolic disorders.[11] Broader self-report measures, such as the International Physical Activity Questionnaire, offer domain-specific detail but are more burdensome and prone to recall bias.[49] The Simple Physical Activity Questionnaire was designed and validated for use in psychiatric populations and may prove preferable.[50] Accelerometry provides objective estimates of PA and SB and has been extensively used in psychiatric cohorts, though cost and feasibility limit routine clinical application.[37] Many people have access to smartphones, and these contain low-grade accelerometers that can prove useful guides for PA monitoring.[55]

PA is generally safe, even for individuals with complex long-term conditions, and the benefits of doing PA outweigh the risks of not doing some for most people.[8,56] Safety screening is an important adjunct for individuals who are at risk, such as those with symptomatic and unmanaged cardiovascular, respiratory disease, or acute mania. The Physical Activity Readiness Questionnaire (PAR-Q+) is widely used to determine need for medical clearance.[52] However, its feasibility in psychiatric populations remains uncertain given literacy, cognitive impairment, and high false-positive rates.[11] Clinicians should weigh risks and benefits, recognizing that for most adults with mental disorders, low- to moderate-intensity PA is safe and does not require medical clearance.[8]

An algorithmic approach may be useful: initial Physical Activity Vital Sign screening, followed by PAR-Q+ if the person is a high-risk candidate (eg, known symptomatic and/or untreated cardiovascular disease) if activity is planned, with detailed self-report or device-based assessment where clinically indicated. Embedding PA and SB assessment alongside vital signs, sleep, and diet provides a foundation for tailored intervention and longitudinal monitoring.[8,23]

Treatment

The consistent evidence for the benefits of PA in SMI has shifted the field from questioning efficacy to focusing on how best to deliver interventions in clinical practice (**Table 3**[8,10,13,17,24,57-62]). A recent umbrella meta-review synthesizing 13 meta-analyses involving more than 12 000 participants confirmed that structured exercise interventions yield moderate to large effects on depressive, anxiety, and psychotic symptoms, as well as improvements in cognition and cardiometabolic health.[13] Similarly, the European Psychiatric Association guidelines on lifestyle interventions highlight PA as a safe and cost-effective strategy that should be systematically integrated into psychiatric care.[8]

The 5A framework (Ask, Assess, Advise, Assist, Arrange)[53] offers a pragmatic structure for implementation and requires minimal training (**Table 4**[8,17,19,43,45,49,50,52,54,64-67]).

Ask

Every consultation should include brief questions on PA and SB, treating these as vital signs. Short tools such as the Physical Activity Vital Sign have been validated for quick screening in busy services.[54]

Assess

For individuals whose PA is below guideline thresholds, more detailed assessment can be undertaken using the Simple Physical Activity Questionnaire[50] or accelerometry where feasible. As noted, the PAR-Q+ can guide whether medical clearance is needed, though

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Table 3. Evidence for PA Interventions Across Major Mental Disorders

Disorder	Evidence from meta-analyses and reviews	Effective modalities	Effect size, SMD	Guideline recommendations	Notes and considerations
Depression	Consistent moderate to large effects on symptom reduction[8,13,17,24,57]	Aerobic, resistance, combined; group or supervised formats most effective	0.5-0.8	WHO[58]: ≥150 min/wk moderate PA; EPA[8,59,60]: PA as adjunctive treatment	Comparable effect sizes to antidepressants; adherence enhanced by supervised and enjoyable activities
Bipolar disorder	Limited but growing evidence; benefits mainly in depressive phases; risks of overactivation in mania[8,13,61]	Aerobic and resistance training; lifestyle-integrated PA	0.3-0.5 (low certainty)	EPA[8,60]: recommend as adjunct where feasible	Monitor for mood destabilization; tailor to illness phase; inclusion of motivational strategies to support PA adoption and maintenance[13]
Schizophrenia and psychotic disorders	Strong evidence for improvements in negative symptoms, functioning, cognition, and cardiometabolic outcomes[8,10,13,62]	Aerobic and resistance, especially supervised programs	0.4-0.6 for symptoms; large effects on fitness and metabolic outcomes	EPA[8]: PA as essential component of care	(None)

Abbreviations: EPA, European Psychiatric Association; PA, physical activity; RR, relative risk; SB, sedentary behavior; SMD, standardized mean difference; WHO, World Health Organization.

Table 4. Practical Clinical Strategies for Implementing the 5A Framework in Psychiatric Care

Step	Key actions	Clinical examples	Metrics to track
Ask[8,54]	Treat PA/SB as vital signs at every visit	"On how many days did you do ≥10 min of activity last week?" "About how many hours a day do you sit?"	PAVS min/wk; reported sedentary h/d
Assess[49,50,52]	Quantify PA/SB; screen safety; identify barriers & preferences; assess stage of change	PAVS/IPAQ/SIMPAQ; PAR-Q+ if initiating MVPA; note passive vs active SB; ask preferred activities and times; Physical Activity Stages of Change Questionnaire[63]	Baseline PA category (meets/doesn't meet guidelines), understand the context of PA (leisure, occupational, travel); PAR-Q+ outcome; barrier checklist
Advise[17,19,64]	Give personalized, diagnosis-sensitive guidance and rationale	"The general recommendation is to aim for 150 min of MVPA per week. This is an aspirational goal. We can start small and build up based on what feels manageable for you." "Break up sitting every 30-60 minutes."	Written/EMR plan issued; patient understanding/teach-back
Assist[50,65,66]	Behavioral support: start with open goals, MI, enable autonomy, competence, relatedness; assist in overcoming barriers before offering to shift to personalized specific goals; FITT principles for those who want to (always considering personal preference for goal type)	"Explore how much you feel like walking each day"; schedule supervised group twice weekly; offer peer/coach support; app/wearable prompts	Foster engagement (weekly); sessions attended; step counts or active minutes; dropout/reasons
Arrange[8,45]	Follow-up, referral, and social prescribing; address SDOH barriers	Review in 2-4 wk; refer to physio/exercise professional; prescribe community walking group; transport voucher	Follow-up kept; referral completed; progression in FITT; reduction in sedentary bouts
Optional: reduce SB[43,67]	Potentially target passive SB; substitute light activity or mentally active SB	Timer to stand every 45 min; swap 30 min TV with a walk; replace doomscrolling with reading or puzzles; PA snacks[68]	Sedentary bout length; breaks/h; time in light PA

Abbreviations: EMR, electronic medical record; FITT, frequency-intensity-time-type; IPAQ, International Physical Activity Questionnaire; MI, motivational interviewing; MVPA, moderate to vigorous physical activity; PA, physical activity; PAR-Q+, Physical Activity Readiness Questionnaire; PAVS, Physical Activity Vital Sign; SB, sedentary behavior; SIMPAQ, Simple Physical Activity Questionnaire; SDOH, social determinants of health.

evidence shows that moderate PA is safe for most individuals with SMI.[11] SB should also be explored, with attention to the distinction between mentally passive behaviors (eg, television viewing or social media use) and mentally active behaviors (eg, reading), which carry different health implications.[43] In addition, assessing readiness for behavior change using tools such as the Physical Activity Stages of Change Questionnaire[69] can help tailor interventions to the individual's motivational state, improving engagement and effectiveness.

Advise

Clinicians should provide clear, personalized advice. International guidelines recommend at least 150 minutes of moderate-intensity PA per week, but framing these as aspirational targets and placing an emphasis on "more is better than some" and "some is better than none" is essential.[8] Approaching in this way may help reduce demotivating people and the risk of unintentionally exacerbating mental health symptoms as a result of shame and stress associated with goal failure.[70] Evidence indicates that both aerobic and resistance training are effective.[17,19,71] PA should be personalized to the capabilities of the individual and ideally a type of PA they enjoy (eg, walking, sport, dancing, yoga). Two days of strength training are also recommended; this can include traditional strength training with weights or using body weight. Clinicians could advise people that while all PA can be helpful, greater mental health benefits are typically garnered from structured PA undertaken for leisure purposes (eg, going for a walk after work, going to the gym, yoga) followed by travel (eg, cycling to work, walking to work) or household activities, compared with work-related PA (eg, walking around a hospital on duty).[72] Supporting changes in PA and SB can facilitate improvements in mental health and should form part of routine mental health provision.[8]

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Table 5. Current Clinical Practice Recommendations and Practical Implications

Topic	Current recommendations (clinician facing)	Uncertainties and controversies	Practical implications and notes
Indication and scope	Offer PA as an adjunct for adults with depression, BD (depressive phase), and schizophrenia; treat inactivity/SB as modifiable risk factors	Relative prioritization vs medications and psychotherapy; optimal timing in care pathway	Present as "core component" that complements standard care; integrate early
Dose and modality	Aim for ≥150 min/wk moderate PA (or ≥75 min vigorous); include aerobic and resistance training; break up prolonged sitting	Exact dose-response by diagnosis; role of HIIT vs moderate continuous; yoga/mind-body positioning	Start low, progress; combine modalities; personalize to preference and symptoms; prioritize leisure time PA
Supervision and setting	Supervised/group formats improve adherence and outcomes in SMI; leverage community programs and social prescribing	Cost-effectiveness across settings; scalability of supervision	Prioritize supervised starts in SMI; transition to blended/community/digital support
Sedentary behavior	Identify and reduce passive SB (TV/scrolling); encourage frequent breaks and light activity substitutions	Optimal "break" frequency/duration; impact of mentally active SB	Use prompts (30-60 min), swap passive SB with light PA or cognitively engaging tasks
Safety and screening	PAR-Q+ or equivalent when initiating MVPA; most individuals safe for moderate PA; monitor meds (eg, β-blockers, antipsychotics)	False-positive results on screening; need for medical clearance thresholds	Do not overmedicalize low-moderate PA; target symptom-informed precautions
Adherence and motivation	Use MI, mental contrasting + implementation intentions; support autonomy, competence, relatedness; peer/coach support; wearables	Best behavior-change package by diagnosis; long-term maintenance strategies	Tailor goals, adopt a graded approach; schedule follow-up; track simple metrics (steps/min), assess preferences, and encourage people to have fun
Phase specific (BD)	Focus on depressive phases; monitor activation; coordinate with mood stabilization	Risk of overactivation/mania with high-intensity PA	Use light-moderate intensity, close monitoring; pause/intensity-adjust during hypomania/mania
Cognition and negative symptoms (SCZ)	Combine aerobic + resistance; supervised, structured programs	Differential effects on cognitive domains; dosing for negative symptoms	Prioritize routine, peer/group formats; add skill-building and role support
Digital/remote delivery	Apps/wearables and brief remote coaching can augment adherence	Data accuracy, privacy, engagement decay	Keep tech simple; use feedback loops; address privacy in consent
Equity and access	Address barriers: transport, cost, stigma; offer low-cost/community options	Best models for low-resource settings; cultural tailoring	Partner with community organizations; provide travel vouchers; adapt culturally
Outcome tracking	Track PA minutes/steps, sedentary breaks, symptom change, fitness markers	Standardized core outcome sets across diagnoses	Use brief core set + 1 disorder-specific outcome; embed in EMR

Abbreviations: BD, bipolar disorder; EMR, electronic medical record; HIIT, high-intensity interval training; MI, motivational interviewing; MVPA, moderate to vigorous physical activity; PA, physical activity; PAR-Q+, Physical Activity Readiness Questionnaire Plus; SB, sedentary behavior; SCZ, schizophrenia; SMI, severe mental illness; TV, television.

Assist

Supporting individuals to set achievable goals and overcome barriers is crucial. One effective approach is to encourage open goals, that is, explorative objectives like "see how active you can be this week," which promote enjoyment, reduce pressure, and enhance motivation, particularly in individuals with fluctuating mental health. Evidence suggests open goals may improve engagement and perceived performance compared with rigid targets.[73] Mental contrasting with implementation intentions can also support behavior change by helping individuals visualize a desired outcome (eg, feeling more energized through regular activity), contrast it with current obstacles (eg, low motivation or fatigue), and then form specific "if-then" plans to overcome those barriers (eg, "If I feel I am too anxious to go to the gym, then I'll follow a short exercise video at home instead"). Combined with motivational interviewing and facilitating autonomy, competence, and relatedness, these strategies can enhance adherence in this population group.[65,74] Group or supervised PA programs are particularly effective in SMI, improving both outcomes and sustained participation.[8] PA facilitators should adopt an instructional style that satisfies individuals' basic psychological needs. We also recommend the use of supervised PA sessions for clinical populations, which appear to be more beneficial than unsupervised sessions.[66] A practical approach may be to focus on modifying 1 behavior at a time, such as breaking up sitting time (eg, stand up every hour), before gradually introducing changes to the second, like increasing PA. Naturally, this strategy should be tailored to the individual's unique circumstances, including their motivation, energy levels, and mood, and as such should be flexible and adapted as needed.

Arrange

Finally, clinicians should organize follow-up and referrals. Embedding trusted and empathetic exercise professionals within mental health teams, or linking to community resources and social prescribing pathways, has been shown to improve sustainability.[45,64] Where this is not possible, supporting access to low-cost or free PA resources locally or using online free PA resources or tracking PA with mobile phones will be important.

Taken together, the evidence suggests PA should be considered a core therapeutic component of psychiatric treatment. Using structured frameworks, clinicians can move beyond efficacy debates and embed PA promotion and SB reduction into everyday care. Embedding exercise specialists into mental health teams has been associated with improved outcomes and acceptable cost-benefit ratios.[8] However, implementation at scale requires workforce training, clear reimbursement structures, and institutional support, which remain major gaps in current psychiatric care systems.[8] There are multiple structural and societal barriers that people with SMI face. Where possible, providing support for access and travel and ensuring inclusion are essential. Addressing these structural barriers is essential for sustainable integration.

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Prognosis

Integrating PA and reducing SB into psychiatric care has the potential to improve prognosis across both mental and physical health domains. Meta-analyses demonstrate that structured exercise reduces relapse risk in depression and psychosis[17,21] and is associated with greater functional recovery, improved cognition, and enhanced quality of life.[8,21,36] Longitudinal evidence further suggests that regular PA reduces all-cause and cardiovascular mortality in the general population, potentially contributing to narrowing of the 10- to 20-year gap in life expectancy.[2,45] SB reduction, particularly replacing mentally passive sedentary behaviors with light or moderate activity, has emerging associations with improved mood and cognitive outcomes.[42,43] Collectively, these findings indicate that embedding movement interventions into standard psychiatric care not only improves short-term symptoms but may confer durable benefits on long-term prognosis and survival. **Table 5** outlines clinical practice recommendations and practical implications for integrating PA and reducing SB into psychiatric care.

Conclusions

Low PA and prolonged SB are major, modifiable contributors to poor outcomes in people experiencing mental disorders. Robust evidence supports PA as a safe, effective, and scalable intervention, yet implementation remains limited. By adopting structured approaches such as the 5A framework, clinicians can embed PA promotion and SB reduction into everyday psychiatric care, improving symptoms, functioning, and physical health while narrowing the mortality gap.

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